October 29, 2026

Paragon Hosts Discussion on the No Surprises Act, Focusing on the Arbitration System.

On October 27th, Paragon Health Institute held an event titled “Fixing the No Surprises Act: Scaling Back and Reforming the Federal Arbitration System”.

Highlights.

- **Arbitration and Independent Dispute Resolution.** Panelists agreed that the federal Independent Dispute Resolution (IDR) process has grown well beyond what Congress expected when it passed the NSA. Dispute volume has risen sharply since 2023, and providers win the large majority of cases. Employer and think tank speakers tied high awards to rising premiums. Provider representatives argued the law works when both parties engage.

- **Emergency vs Scheduled Procedures.** The NSA was designed mainly to protect patients receiving emergency care they could not plan around. Scheduled services such as surgery now account for about 25 percent of award dollars. Gelfand stressed that the law was never meant to cover scheduled care. Pilgrim cautioned that emergencies can arise even during scheduled procedures and that those patients still need protection.

Additional details on the event are below. As always, please feel free to reach out with any questions.

Panel One

- Loren Adler, Senior Fellow of Economics & Associate Director of Center on Health Policy, Brookings Institute
- Ben Ippolito, Senior Fellow, American Enterprise Institute
- Jackson Hammond, Senior Policy Analyst, Paragon Health Institute
- Brian Blase, PhD President, Paragon Health Institute (*Moderator*)

Independent Dispute Resolution/Arbitration
Blase asked Ippolito what Congress was trying to accomplish with the No Surprises Act (NSA). Ippolito explained that patients were being unexpectedly treated by out-of-network providers, usually in emergencies, and needed more leverage against surprise bills. Hammond noted that merit line items in dispute grew from 258,072 in 2023 to 1,618,586 in 2024 and 1,184,140 in 2025, while provider win rates climbed from 82 to 85 percent. He explained that median awards track median provider offers and that insurers lose even when they raise their offers. As an example, he pointed to Cigna, which offered more than other insurers but still lost most of its disputed line items. Adler said the system has expanded beyond what Congress intended. He noted that about 25 percent of award dollars come from surgery and that awards run about 2x the expected level for emergency medicine and 5 to 7x for surgery.

Arbitration vs Benchmark
Blase asked why Congress chose arbitration. Ippolito explained that lawmakers also considered a benchmark approach, but saw it as explicit rate setting and inflexible. He added that arbitration, by

contrast, was viewed as better able to weigh each provider's intrinsic value. Adler argued this is a misconception, since arbitration still sets an out-of-network price, which was expected to land slightly above median in-network rates. He cautioned that the third-party process is opaque and tends to favor doctors. Ippolito added that the outcome matters far more than the win rate and that the process' vagueness is not creating a better market. He noted that even though most eligible claims never reach IDR, high awards still drive up rates.

Solutions
Hammond proposed two tracks: (1) require upfront prices so patients know costs in advance; and (2) cap emergency payments at a set percentile of regional in-network rates, which he said insurers could not object to since they negotiate those rates. Adler called for strong rules ensuring patients know which doctors are in network and for requiring all doctors at an in-network hospital to share the same network, though he acknowledged this complicates emergency care. Ippolito supported a bundled approach that places physicians' bargaining power with the institution, though he doubted it would happen. He pointed to newly available price transparency data as a more likely route to eliminating ghost rates.

Political Outlook
When asked how the politics around arbitration have shifted, Hammond said rising premiums will push Congress to act. Adler noted that doctors' concentrated interests make change hard to push, and Ippolito said meaningful reform is unlikely.

Panel Two

- James Gelfand, Chief Executive Officer and President, ERISA Industry Committee (ERIC)
- Randy Pilgrim, MD, Emergency Physician; Chief Medical Officer for SCP Health; Chair, EDPMA Federal Health Policy Committee
- Ryan Long, Director of Congressional Relations & Senior Research Fellow (*Moderator*)

Emergency vs Scheduled Procedure
Long asked how the NSA should treat emergency versus scheduled procedures. Gelfand stated that the NSA was never meant to cover scheduled care, noting that a waiver process existed for those cases. Pilgrim countered that emergencies can arise even during scheduled procedures and that patients in those moments are especially vulnerable and still need protection.

Shared Savings Program
Long asked whether shared savings programs are a real emerging trend. Gelfand urged Congress to ban them, arguing they are becoming inflationary as they become part of the NSA landscape. He acknowledged that a ban alone would not fix the broader issue, which he said requires free market solutions. Pilgrim also called for a ban, explaining that these programs hold providers back from getting in network.

Health and Human Services New Gateway
Long asked how HHS' new gateway could help reduce the number of ineligible claims. Gelfand said he hopes it will clarify what is happening in IDR, noting it is unclear why certain IDR entities (IDREs) receive certain cases. He called for changing how IDREs are paid, since their pay is tied to adjudicating claims. Pilgrim said he expects the gateway to be very helpful.

Eligibility Issues

Pilgrim argued that the NSA needs refinements around eligibility. He said jurisdictional eligibility could work well with the right information, since those cases can be identified and kept out of the NSA process. He added that procedural eligibility can work as an open-book test that everyone can check. He said employers should not be paying ineligible claims, but the solution is to fix eligibility, because people should pay their bills.